Loop Health Memo

Overview

Loop Health is an AI-native health insurance broker in India, paired with virtual primary care. The company covers 1.1 million lives and placed ₹672 crore in gross written premium (GWP) in 2025, having doubled revenue every year for the past five years. This growth has catapulted Loop from the 200th largest health insurance broker in India in 2021 to the 8th largest today — a ranking where the only peers ahead of them at the top are multinational brokers like Marsh, Aon, and Willis.

Loop has raised $60 million to date from General Catalyst, Khosla Ventures, United Health, and Elevation Capital. The company is now at an inflection point: it has a signed term sheet to acquire a standalone health insurance carrier license from the Hinduja Group for $28M — one of India's nine largest family conglomerates — and plans to begin placing risk on its own carrier as early as August 2026.

Founding Story

Loop's origin is personal. In 2019, Mayank (CEO) had to leave the University of Illinois mid-semester after his father suffered a heart attack and required bypass surgery. When Mayank later brought his father to see specialists in the US, he discovered that the surgery likely wasn't necessary. This set him on a year-long investigation: he spoke to over 150 doctors across India trying to understand how this could have happened so routinely.

What he found was a structurally broken system. Kickbacks between referring physicians and hospitals are not illegal in India, and most GPs and specialists generate roughly half their income through such arrangements. According to former CEOs of top-five Indian hospital systems, an estimated one-third of surgeries performed in India are medically unnecessary. Mayank's father was not an outlier — he was the norm. Another shocking statistic is that the C-section rate in India is 70%+ vs global averages of 15%.

Mayank studied the integrated payvider models that had addressed similar dynamics in other markets: Kaiser Permanente, Geisinger, United Health / Optum, and PingAn in China. The largest insurers in each of these markets are integrated payviders- this model has won globally. He founded Loop to bring that playbook to India.

Why India, Why Now

India is the fastest-growing healthcare market in the world, estimated at $150–250 billion and expanding at over 20% annually — a rate roughly twice that of China or Brazil. Yet health insurance accounts for only about 10% of total healthcare spend today, compared to 85% in the US. This is not a sign of structural weakness; it is a sign of how early the market is. Health

insurance only emerged as a meaningful product in India relatively recently, and the $14 billion group health insurance market is growing at 20% per year according to IRDAI, the national regulator.

The dominant channel today is employer-provided group health insurance, which covers approximately 250 million lives, while retail covers only 50 million. The government mandated employers to provide health insurance in 2020.

Coverage itself is also rapidly expanding: three years ago, fewer than 5% of employers covered annual health checkups; that figure has grown to 30%. GP visits, diagnostics, and specialist care are all beginning to be covered where none existed before. India's health insurance market today looks very much like the US market in the 1960s — on the cusp of a structural expansion in both coverage and complexity.

More details on our market thoughts here.

The Four-Step Playbook

Loop was designed from the start to become India's first integrated payvider — a company that simultaneously owns the insurance product, the primary care front door, and eventually the specialist and hospital relationships. The company has been executing a deliberate four-phase strategy since inception.

Phase 1: Distribution

Loop entered the market as a broker because a brokerage license is relatively easy to obtain and distribution, once established, becomes an extremely durable competitive asset. The goal was to reach 10 million lives as quickly and capital-efficiently as possible. The company is already at 1 million lives and doubling annually.

Phase 2: Primary Care Bundled with Insurance

Loop differentiated its broker offering by bundling virtual primary care directly into the insurance product. This was strategically important for two reasons. First, primary care is the gateway to the entire healthcare system — the front door through which patients are routed to specialists, diagnostics, and eventually surgeries. Owning that front door is how you steer care. Second, standard health insurance in India does not cover primary care visits, yet nearly everyone gets sick at least once a year while only 8% of lives make a formal claim.

Loop members get access to a care team that responds in 30 seconds 24x7, and unlimited virtual consults for the year. Loop's healthcare engagement has exceeded 250,000 interactions in the last twelve months against an employee base of 500,000, demonstrating genuine, habitual utilization.

The market response has been decisive. Loop's win rates are five times the industry average, and the bundled offering is what has driven the company from 200th to 8th largest broker in just five years.

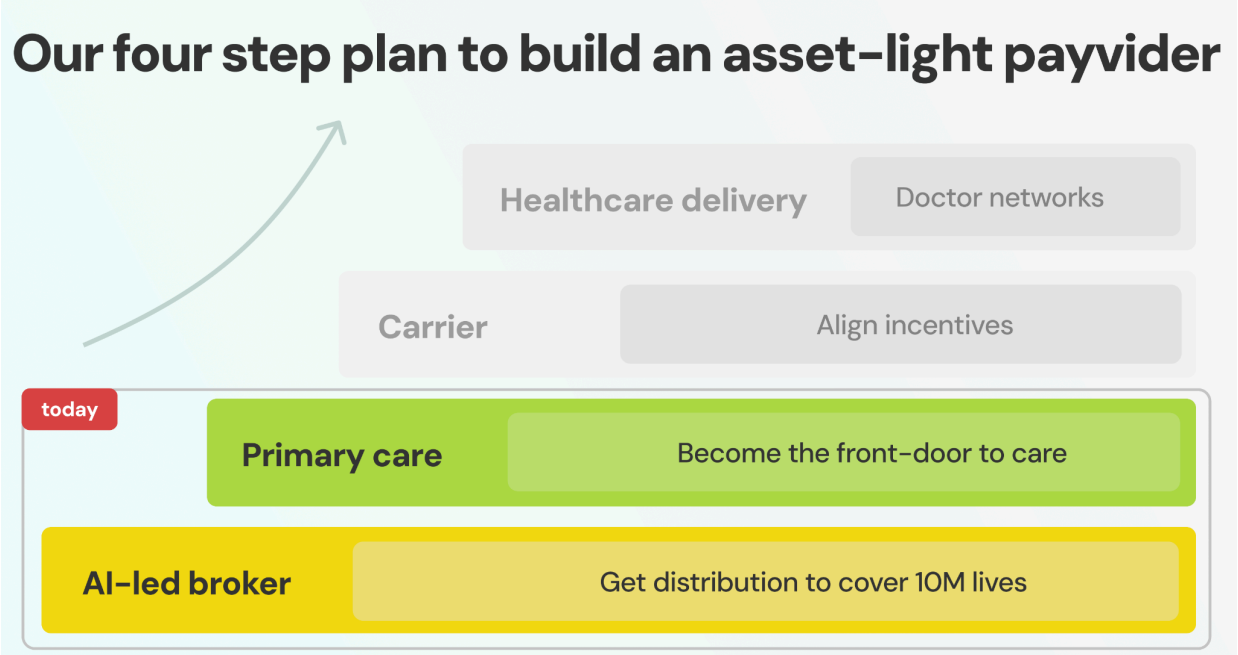
Phase 3: Becoming the Carrier

With millions of lives concentrated across five cities and a proven primary care relationship, Loop is positioned to own the insurance dollars flowing to hospitals — and with that ownership, to fundamentally reshape provider incentives. The company built its own technology stack from scratch, giving it capabilities no legacy carrier can match: claims can be paid in a single day, and differentiated OPD products can be built and iterated on rapidly. Other carriers are running on IT infrastructure operated by firms like TCS, with legacy systems that cannot be refactored.

There are only seven standalone health insurance licenses in India, and every one of them has become a multi-billion dollar company. Loop has a signed term sheet to acquire the license held by the Hinduja Group and expects to begin underwriting its own risk in August 2026.

Phase 4: In-Person Clinics and Specialist Networks

With 250,000 lives per city doubling annually, and with the insurance float to fund capital expenditures, Loop's terminal ambition is to build high-tech primary care clinics that insurance actually covers — something no one has done in India. This creates a closed-loop system: members access primary care through Loop's clinics, Loop directs referrals to its own specialist and surgeon network, and Loop's AI tools monitor prescriptions and care decisions in real time to prevent unnecessary procedures. Loop will deploy AI to listen to consultations, flag overprescription, and equip doctors with clinical decision-support copilots that align incentives with patient outcomes rather than surgical volume.



Structural Advantages of Indian Health Insurance

Indian health insurers operate in a structurally superior economic environment compared to their US counterparts. Medical loss ratios in India average 65% and are unregulated, versus a mandated 85% in the US. Premiums are paid annually in advance and sit in bonds yielding approximately 7%, compared to 4.5% in the US. The combined effect is a 10% net profit margin for Indian carriers versus 2.5–3% in the US, with roughly half of Indian net profits generated from the float business. This is not a temporary dynamic — it is a function of regulatory structure and capital market conditions that are unlikely to converge with the US in the near term.

As a result, Indian health insurance carriers trading with 20–30% growth command 3–4x GWP multiples in the public markets. Private transactions involving companies growing at rates closer to Loop's have traded at 6–10x premiums.

Loop's Competitive Position

Loop enters the carrier market with **three structural advantages** that will be hard for a legacy insurer to replicate.

The first is AI-driven admin reduction. Insurance is a structurally low-margin business with a disproportionately high administrative burden — which makes it, as Loop astutely notes, one of the categories most likely to be transformed by AI. The company has already demonstrated this in practice: cost per claim is down 30% and cost per endorsement is down 50% in 2025 alone, with an increasing share of claims now processed entirely by LLMs without human intervention. The roadmap from here is to **compress opex from 16% of premium down to 10%**, a reduction that would deliver a meaningful and durable margin advantage over every legacy carrier in the market.

The second lever is owned distribution. Because Loop built its carrier business on top of a broker, it controls its own sales force — something no other Indian insurer can claim. Today, insurers across the market pay out roughly 12% of premium annually in broker commissions. For Loop, that variable cost becomes a fixed one over time as the carrier scales. The company estimates it can capture approximately half of that commission expense as permanent savings, **adding around 5 percentage points of gross profit** directly to the bottom line.

The third area is healthcare services revenue. Loop is already monetizing this channel: 15% of healthcare interactions result in a lab referral, split roughly evenly between employer-paid and member cash-pay. As Loop deepens its presence in single-specialty care — and as employer coverage continues expanding into diagnostics, pharma, and specialist visits — the opportunity to follow the United Health / Optum playbook becomes increasingly concrete. A conservative estimate puts **incremental healthcare services margin at 5%** of GWP flowing to the bottom line.

More details on our cost advantage can be found here.

Valuation and Return Potential

By 2028, Loop projects $200 million in GWP on its own carrier, growing at 100% year-over-year — a projection grounded in the fact that it is already at $80 million in GWP on the broker, also growing at 100% annually. We will move over just 30% of the broker book to the carrier in the first few years of the business.

Applying a conservative 6.5x GWP multiple to the carrier implies a $1.3 billion valuation in 2028 from the insurance business alone. The healthcare services segment is projected to reach $30 million in revenue by 2028, growing at 140% annually; valued at 10x revenue (below private market comparables, which trade at 20x for similar growth profiles), this contributes an additional $300 million. Assuming no value is attributed to the broker itself, the blended valuation reaches approximately $1.6 billion — implying roughly a 3x return from an investment made today at a $500 million pre-money.

At IPO, targeted for 2031, Loop projects approximately $850 million in premium growing at 40%, implying a $3.8 billion carrier valuation at 4.5x GWP. Healthcare revenue is projected at $150 million, contributing another $1 billion at 7x. The combined valuation of approximately $4.8 billion, net of dilution from the Series C and IPO, implies an 8x return on capital invested today.

Upside case

Beyond the base case, if Loop executes on its competitive positioning, we could more than double the return profile outlined above.

Those levers would move Loop's net profit margin from the 10% baseline of a traditional Indian health insurer to approximately 25% — more than doubling the return projections in the base case above.